

Synthetic Patient Medical Record

FOR EDUCATIONAL / SOFTWARE TESTING PURPOSES ONLY - NOT A REAL PATIENT

Patient Name	Jordan Taylor (Synthetic)	Date of Visit	August 8, 2026
Age	58	Sex	Male
Record ID	SYN-000184	Provider	Dr. Avery Morgan

Chief Complaint

Patient presents with increasing shortness of breath, fatigue, and intermittent coughing for the past three days. Symptoms are worse with physical activity. Patient denies chest pain, loss of consciousness, or recent injury.

History of Present Illness

The patient reports that symptoms began gradually three days ago. He has noticed mild wheezing, especially at night, and becomes winded while climbing stairs. He reports no known recent exposure to illness. Oral fluid intake has been slightly decreased. He has not experienced fever, chills, vomiting, or significant swelling of the legs.

Past Medical History

Hypertension diagnosed approximately eight years ago. Type 2 diabetes mellitus diagnosed five years ago. Seasonal allergic rhinitis. No previous history of myocardial infarction, stroke, or chronic lung disease.

Current Medications

Metformin 500 mg by mouth twice daily with meals. Lisinopril 10 mg by mouth once daily. Cetirizine 10 mg as needed for seasonal allergies.

Allergies

No known drug allergies.

Vital Signs

Temperature: 98.7 F. Blood pressure: 148/88 mmHg. Heart rate: 96 beats per minute. Respiratory rate: 20 breaths per minute. Oxygen saturation: 94% on room air.

Physical Examination

Patient is alert, oriented, and able to speak in complete sentences. Mild bilateral wheezing is present on lung examination. Heart rhythm is regular without an obvious murmur. No significant lower-extremity edema is observed. Skin is warm and dry.

Diagnostic Results

Chest x-ray shows no focal pneumonia or acute cardiopulmonary abnormality. Basic metabolic panel is within expected limits except for mildly elevated glucose at 162 mg/dL. Complete blood count does not show significant leukocytosis.

Assessment

Shortness of breath with mild wheezing. No evidence of focal pneumonia on chest imaging. Hypertension and type 2 diabetes remain part of the patient's active medical history.

Plan

Continue current home medications. Begin albuterol inhaler as needed for wheezing according to provided instructions. Encourage adequate hydration and rest. Follow up with the primary care provider within one week for reassessment. Patient was instructed to seek urgent medical attention for worsening shortness of breath, chest pain, confusion, blue discoloration of the lips or skin, or other rapidly worsening symptoms.

Follow-Up Note

Patient verbalized understanding of the plan and follow-up instructions. Patient left the visit in stable condition.

Testing note: All names, identifiers, clinical details, and events in this document are fictional and were created solely to test a medical-record summarization application.